

건 명	헤파빅주 인서트(말레이시아 등록용)		
규 격 (가로x세로)	100x250mm	Q A	오창해사팀
		표시문안	7,5pt
색 상	먹 1도	후 가 공	
업체제작일	2023.01.19.	작 업 처	(주)트리스톤



칼선 PANTONE 185C

※인쇄진행시, 인쇄사양과 다를경우
녹십자로 즉시 연락주시기 바랍니다.

Hepatitis B Immune Globulin, Human

Hepabig

Description

Hepabig is a sterile solution containing Hepatitis B immune globulin (HBIG) which is prepared by Cohn fractionation method from plasma of individuals with high titers of anti-HBs and whose plasma does not show serologic evidence of HBsAg. Also, donor units found to be negative to anti-HIV 1/2 and anti-HCV are only used in the preparation of Hepabig.

Composition

	0.5 ml	1 ml
HBIG, Human	100 I.U.	200 I.U.
Aminoacetic acid	11.25 mg	22.5 mg
Sodium chloride	2.5 mg	5.0 mg
Water for injection	q.s.	q.s.

Indications

- For prophylaxis of hepatitis B after exposure to HBsAg, e.g., by accidental "needle-stick" or direct mucous membrane contact by accidental splash, or oral ingestion (pipetting accident) involving HBsAg positive materials such as blood, plasma or serum.
- For prophylaxis of hepatitis B in neonates (In principle, it is jointly used with precipitated Hepatitis B vaccine).

Dosage and Administration

- Following exposure to HBsAg
 - The recommended dose for adult is 1,000 - 2,000 I.U. intramuscularly and if necessary, the dose should be increased or repeated.
 - In children, inject 32 - 48 I.U. per kg of body weight.
 - Hepabig should be administered within 7 days after exposure to HBsAg (preferably within 48 hours).
- In neonates
 - The recommended initial dose is 100 - 200 I.U. intramuscularly.
 - The first dose should be administered within 5 days after birth (preferable within 12 hours), and booster dose should be 32 - 48 I.U. per kg of body weight.
 - The booster dose should be administered between 2 and 3 months after the first administration.

Contraindications

Hepabig should not be administered to following individuals;

- HBsAg positive individuals (but in neonates, Hepabig may be administered regardless of testing results of HBsAg)
- Individuals with anaphylactic reaction to this drug or components of this drug
- Individuals with medical history of shock reaction to the components of this drug

Precautions

- Hepabig should be administered very carefully to patients with IgA deficiency syndrome.
- Hepabig should be administered intramuscularly only and not intravenously.
- In neonates who are administered one dose of 1 ml, Hepabig is injected at two different sites (0.5 ml/site).
- Do not reuse or save for future use. This product contains no preservative; therefore, partially used vials should be discarded

immediately.

Adverse Reactions

Adverse reactions are infrequent, mild and transient following administration of HBIG.

- Local pain, swelling and induration may occur at the injection site.
- Rash and fever may also occur occasionally.

Drug Interactions

- Antibodies present in HBIG may interfere with the immune response to live virus vaccines such as measles, mumps and rubella.
- Vaccination with live virus vaccines should be deferred until approximately 3 months after administration of HBIG.
- Revaccination to persons who received HBIG is necessary shortly after live virus vaccination.
- Hepatitis B vaccine may be administered at the same time, but at a different injection site, without interfering with the immune response.
- No interactions with other products are known.

Storage and Expiry Date

- Store at 2 - 8°C. Warning: Do not freeze.
- Maximum validity : 30 months from the manufacturing date

How supplied

0.5 mL / vial × 1, 5, 10
1 mL / vial × 1, 10, 50

Name and Address of Product Registration Holder in Malaysia.

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